

Media Information

From: Main Street Radiology To: Page: 1/2 Date: 9/5/2025 4:45:41 PM



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Glendale 83-14 Cooper Ave, Glendale, NY 11383

• HUI WEN HUANG, NP
• 135-27 38th Ave, 2nd FL
FLUSHING, NY

Referrer Phone:
Name: FENGMEI SUN
Phone:
Gender: Female
DOB: 3/4/1954
Patient:
Exam Date: 9/3/2025 12:41 PM

Acc #: 10084459
Secondary ID:
Exam Name: |

EXAMS: US BREAST BIOPSY, MA POST BIOPSY MAMMO

HISTORY: 71 years old female presents with a hypoechoic mass in the left breast. The patient presents for ultrasound guided core biopsy.

PROCEDURE: Ultrasound guided core biopsy of the left breast

Pertinent imaging studies and/or documents were reviewed prior to the procedure. Patient identification and procedure were verified. After risks, benefits and alternatives to the procedure were discussed with the patient, agreed to the procedure and written informed consent was obtained. Time-out was performed.

Targeted ultrasound redemonstrates a 0.6 cm hypoechoic mass in the left breast, 3:00 axis, 4 cm the nipple. The lesion was targeted for ultrasound-guided biopsy.

The patient the procedure using standard aseptic technique. Local anesthesia was achieved with injection of 5 cc of 1% lidocaine in the skin/superficial tissues and the deeper tissues. A 14-gauge spring activated biopsy device was advanced to the preselected target through a 13-gauge coaxial guide. Biopsy specimens were obtained, with pre- and post-fire images documenting needle placement for each pass. The tissue specimens were submitted to the pathologist.

A rod shaped clip marker was then deployed at the biopsy site.

The breast tissue is heterogeneously dense, which may obscure small masses. Post procedure digital mammogram discloses the rod shaped marker at the expected location.

The patient site of biopsy was held in compression for several minutes and the needle entrance site was closed using Steri strips. Pain control was adequate and the patient was assessed and appropriate for discharge. Home care instructions for this procedure were reviewed with the patient.

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Continued...

Main Street Radiology

Name:

Date of Birth: 3/4/1954

IMPRESSION:

Ultrasound guided core biopsy of hypoechoic mass in the left breast, 3:00 axis.

Final pathology results:

Ductal carcinoma in situ. DCIS of the papillary and cribriform form type with intermediate grade nuclei. Surrounding breast tissue shows calcifications.

Pathology shows malignant findings. The results are concordant with imaging.

RECOMMENDATION:

Surgical consult/excision is recommended. A bilateral breast MRI with contrast may also be considered to evaluate for extent of disease. Localization of the rod shaped clip can be performed prior to excision.

BI-RADS CATEGORY: 6 - Known Malignancy - Biopsy Proven

RECOMMENDED FOLLOW UP: *Appropriate surgical and oncological evaluation is recommended.*

These findings were communicated to NP Huang's office on

Electronically signed by: Alicia DeRobertis MD

1:02 PM

Workstation: NYPQDEROB

Report Electronically Signed by: Alicia DeRobertis MD

Report Electronically Signed on: 01:02 PM

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Document Information

Other: Outside Procedure

US Bx Procedure Report & Post MMG

00:00

Attached To:

OUTSIDE PROCEDURE ORDER

Orders Only on with Historical Provider,

Source Information

Rea Kate Toy | Msk Pas